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TCGA-BL-A33M-01A-PR Redacted



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SURGICAL PATHOLOGY REPORT

SPECIMEN

- A. Left distal ureter
- B. Right distal ureter
- C. Bladder prostate
- D. Urethral margin
- E. Right pelvic lymph node
- F. Left pelvic lymph node
- G. Appendix

1CD-0-3
carcinoma, urothelial, NOS 8/20/3
Site: bladder, NOS C67.9
lw
12/8/11

CLINICAL NOTES

CLINICAL HISTORY: Bladder cancer.

FROZEN SECTION DIAGNOSIS

- A. Benign.
- B. Benign.

DFS: Urethral margin, biopsy - No dysplasia or malignancy.

GROSS DESCRIPTION

A. Received fresh in a container labeled "left distal ureter" is a portion of tan-white tubular tissue with minimal surrounding adipose tissue, 0.6 cm in diameter and 0.3 cm long. This is entirely sent for frozen section in one block.

B. Received fresh in a container labeled "Right distal ureter" is a portion of tan-white tissue with surrounding yellow adipose tissue, 0.5 x 0.4 x 0.3 cm. It is entirely submitted for frozen section in one block.

C. Received fresh, subsequently fixed in formalin, labeled "bladder and prostate" is a bladder and prostate. The specimen has a total weight of 358 grams and overall dimension of 10.5 x 10.5 x 4.0 cm. The prostate is 4.5 x 3.5 x 3.5 cm. and the bladder is 6.0 x 6.0 x 4.0 cm. The prostate area is heavily stapled. Vasa deferentia average 9.0 x 0.5 cm. The left ureter is 4.0 x 0.5 cm. and the right ureter is 4.5 x 0.5 cm. The bladder has a pink tan smooth glistening wrinkled mucosa. There is a 1.3 x 1.3 cm. ulceration in the right wall, 1.7 cm. from the trigone and 5.0 cm. from the prostatic urethral margin. The cut surface of this area is firm, yellow, and rubbery, with possible scarring and possible tumor, which is 0.6 cm. in greatest dimension and comes within 1.2 cm. of the closest surgical margin which is inked blue. The remainder of the mucosa is pink tan smooth glistening, showing no other discrete gross lesions. Both ureteral orifices are probe patent. No lymph nodes are identified in the surrounding fatty tissue. The right half of the prostate is inked blue and the left half inked black. The cut surface of the prostate is pink tan and nodular, showing no other discrete lesions. Representative sections of the specimen are submitted as follows:
Block 1 - prostatic urethral margin en face; block 2 - prostatic urethra and posterior aspect of prostate; block 3 - trigone to prostate; block 4 - representative anterior bladder; block 5 - representative dome of bladder; block 6 - representative posterior bladder wall; block 7 - right side ureteral margin, vas deferens

margin and seminal vesicle; block 8 - left side vas deferens margin, left ureteral margin and seminal vesicle; block 9 - left side bladder with ureteral orifice; block 10 - right side bladder with ureteral orifice and portion of ulceration; block 11,12 - full thickness composite with ulceration and possible tumor; blocks 13,14 - full thickness composite with ulceration and tumor; block 15 - full thickness ulceration; blocks 16-18 - additional representative sections; 19 - full thickness ulceration; block 20-21 - prostate, full thickness composite; block 22-23 - prostate, mid to posterior aspect composite; blocks 24-25 - prostate, mid to posterior aspect composite (21 through 25 are of prostate, from apex to base).

D. Received is a container labeled with the patient's name, medical record number and "urethral margin". The specimen consists of two fragments of pink-tan tissue measuring 1 x 0.8 x 0.5 cm in aggregate. AS

E. Received fresh, subsequently fixed in formalin labeled "right pelvic lymph node" is a 5.5 x 3.5 x 1.2 cm. aggregate of yellow lobular fatty tissue fragment which may contain possible lymphoid tissue present. Lymph nodes range from 0.3 cm. to 3.0 cm. in greatest dimension. The tissue fragments are entirely submitted in four cassettes. AS4.

F. Received fresh, subsequently fixed in formalin labeled "left pelvic node" is a 4.5 x 3.5 x 1.5 cm. aggregate of yellow pink lobular fatty tissue which may contain possible lymphoid tissue. The tissue fragments may contain lymphoid tissue and are entirely submitted in four cassettes. AS4.

G. Received fresh, subsequently fixed in formalin labeled "appendix" is a grossly unremarkable appendix which is 10.0 x 0.7 cm. The serosa is pink tan smooth glistening partially covered with abundant yellow lobular fat. No fibrinous exudate, hemorrhage or necrosis is grossly identified. Representative sections of the specimen are submitted in one cassette with the proximal end inked. RS1. (proximal end inked)

MICROSCOPIC DESCRIPTION

Histologic type: Urothelial carcinoma.
Histologic grade: High-grade.
Associated epithelial lesions: Carcinoma in-situ.
Primary tumor (pT): pT3 (Carcinoma invades through the muscularis propria and into the perivesicular fibroadipose tissue).
Margins of resection: Negative
Regional lymph nodes (pN):
Four negative right pelvic lymph nodes (0/4).
Four negative left pelvic lymph nodes (0/4).
Vascular invasion: Present (slide C19).
Direct extension of invasive tumor: Not identified.
Other findings: Prostate uninvolved by carcinoma (nodular hyperplasia); unremarkable appendix.

5, 4x2, 3x3, 1, 14x3

DIAGNOSIS

- A. Ureter, left distal margin, excision:
Negative for malignancy or dysplasia.
- B. Ureter, right distal margin, excision:
Negative for malignancy or dysplasia.

- C. Bladder and prostate, radical cystectomy and prostatectomy:
 High-grade urothelial carcinoma, with invasion through the
 muscularis propria into the perivesicular fat and soft tissue.

Angiolymphatic invasion identified.
 Prostate uninvolved by carcinoma.
 Negative margins of excision.

- D. Urethral margin, biopsy:
 Negative for malignancy or dysplasia.
 E. Lymph nodes, right pelvic, excisional biopsy:
 Four negative lymph nodes (0/4).
 F. Lymph nodes, left pelvic, regional resection:
 Four negative lymph nodes (0/4).
 G. Appendix, incidental appendectomy:
 No specific pathologic abnormality.

MD (Electronic Signature)

--- End Of Report ---

Criteria	Yes	No
Diagnosis Discrepancy		/
Primary Tumor Site Discrepancy		/
HIPAA Discrepancy		/
Prior Malignancy History		/
Dual/Synchronous Pathology Noted		/
Case Is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	W 12/8/11	